An Open Letter to Healthcare Professionals

I'm a scientist by training, but not in clinical medicine. My work revolves around **research, mechanistic modeling, and publishing in peer-reviewed journals**. Because of my background, I'm comfortable with **mechanistic extrapolation**: understanding how a biological system works, proposing an intervention, and predicting how that system might respond. When the reasoning is careful and grounded in good evidence, those predictions can be useful. But biological plausibility is not clinical proof.

And that's not how the FDA approves drugs.

FDA approval requires sufficient clinical evidence that a treatment's benefits outweigh its risks for an intended use. I respect that standard. It's an essential protection against ineffective and dangerous therapies.

But our system has a significant failing: **many potentially safe and effective treatments may never receive the investment needed to demonstrate their benefits clinically.**

- If a compound doesn't promise sufficient financial return, the trials may never be funded.
- If intellectual property protections cannot support commercial exclusivity, the economic incentive to conduct expensive trials may disappear.
- If a compound primarily improves **healthspan, physiological resilience, or aspects of healthy aging**, demonstrating an approvable clinical benefit becomes especially challenging. Aging itself lacks a conventional FDA-approved therapeutic indication, and measuring meaningful improvements in healthspan requires difficult, expensive, long-duration studies.

This leaves a growing category of **peptides, small molecules, and biologics** with promising mechanistic evidence, preclinical findings, and sometimes encouraging early human results, but without the large clinical trials needed to establish their broader therapeutic value.

Lack of approval doesn't mean a compound is ineffective. But it also doesn't establish that it's safe.

That gap leads some people like me to undertake **careful, informed N=1 experimentation**.

I take that responsibility seriously. Before injecting an investigational peptide, I conduct a **deep dive into the published literature**: in vitro research, animal studies, human trials when available, pharmacokinetics, and mechanistic reviews. I look for safety signals,

biological plausibility, reasonable exposure ranges, and evidence that supports or contradicts my hypotheses.

I recognize that **promising mechanistic evidence does not eliminate unknown human risks**. Missing pharmacokinetic data and uncertain long-term safety are significant challenges. Careful research can inform decisions, but it cannot make those uncertainties disappear.

Here's what my personal experimentation looks like:

- **Retatrutide for metabolic health.** My continuous glucose monitoring shows substantial improvements in glycemic stability. Human trials have demonstrated impressive effects on weight and glycemic control, while additional cardiovascular and metabolic benefits remain under investigation. I believe incretin-based treatments deserve a much larger role in preventing metabolic deterioration before patients develop advanced disease.
- **Tesamorelin morning and evening**, which I cycle 12 weeks on and 6 weeks off. The clinical evidence for visceral fat reduction and improved body composition is substantial. My own observations suggest improvements in sleep, recovery, and physical function.
- **Thymulin five days weekly and thymosin α1 twice weekly** for immune modulation. Immune benefits are difficult to perceive directly. But after beginning thymulin, a chronic overnight postnasal drip I'd experienced for years disappeared.
- **GHK-Cu daily for tissue repair and KPV daily for inflammatory modulation.** Both have interesting mechanistic and preclinical evidence, although their systemic benefits in humans remain much less established.
- **Thymosin β4 and BPC-157**, cycled two weeks on and two weeks off. I perceive noticeably improved recovery following demanding resistance workouts or heavy yardwork. Those observations don't prove efficacy, but they are consistent with some of the tissue-repair mechanisms explored in preclinical research.
- **ARA-290 in a staggered weekly protocol** for potential small-fiber nerve protection, microvascular resilience, and inflammatory modulation. Controlled human studies in specific neuropathic conditions provide encouraging signals, while broader healthspan applications are my own extrapolation.
- **Metformin and tadalafil**, used for potential healthspan benefits beyond their established indications. Both have extensive human clinical experience, and the aging-

related literature is compelling enough, in my view, to warrant further investigation for otherwise healthy older adults remains an open question.

I'm not suggesting these choices are appropriate for everyone, or even anyone else but me. **My observations are observations, not proof of efficacy.** Nor am I suggesting that every peptide is promising or that every experiment is sensible.

Some compounds have meaningful safety concerns. **Melanotan II** has concerning reports involving pigmented lesions and melanoma, although causality remains uncertain. GH-axis therapies can elevate IGF-1, alter glucose metabolism, and produce other clinically important effects. Peptides affecting angiogenesis, including thymosin β4 derivatives, raise theoretical questions about malignancy that deserve careful consideration. Other compounds, such as DSIP, have inconsistent efficacy evidence. And SS-31 (elamipretide) illustrates how a peptide can produce mixed results in some clinical populations while eventually receiving FDA approval for a specific indication.

The biology matters. The evidence matters. The uncertainty matters.

And that brings me to the purpose of this letter.

Your patients are already using these compounds.

Some of your patients are using **BPC-157, TB-500, GHK-Cu, KPV, retatrutide, and other investigational peptides**. More will be doing so in the coming years.

Some are thoughtful, careful, research-driven individuals who understand the uncertainties and attempt to manage them responsibly. Others may be inexperienced, following recommendations from social media, bodybuilding forums, or unreliable suppliers.

Both groups need informed healthcare partners.

The medications, laboratory tests, diagnoses, and treatments you recommend should take into account what your patients have chosen to do, even when you disagree with those choices.

For that to happen, patients must feel **safe telling you everything they're taking**.

They won't disclose their experimentation if they expect to be dismissed, ridiculed, or lectured.

It's entirely appropriate to say:

"This compound is not FDA-approved for your intended use. I can't recommend it because we don't have sufficient clinical evidence to establish its safety and efficacy."

But please consider adding:

"Thank you for telling me. I'm not judging you. I want to understand what you're taking so we can make informed decisions about your healthcare."

That acknowledgment can make an enormous difference.

You don't need to become a peptide specialist.

A little peptide awareness can make you a more effective clinician for patients who are already experimenting.

You don't need hours of specialized research to get oriented. An AI research assistant can help summarize the published clinical and preclinical literature on BPC-157, thymosin β4, GHK-Cu, KPV, retatrutide, or other compounds.

The practical implications can be straightforward:

If a patient uses **GH-axis peptides such as tesamorelin, ipamorelin, or CJC-1295**, consider appropriate IGF-1 and glucose monitoring, along with potential endocrine risks.

If they're using **thymosin β4 or products marketed as TB-500**, understand the distinction between the full-length peptide and its fragments, and discuss the uncertainties surrounding angiogenesis and malignancy; consider deeper cancer montoring.

If they mention a **"GLOW blend,"** ask exactly what's in it. Commercial formulations vary, and the name alone doesn't establish its ingredients.

You don't have to endorse their experimentation to help make it safer.

I have what I consider a model relationship with my primary care physician. I tell her everything. **Every peptide I use is documented in my electronic health record.** We discuss my research, my observations, and what additional laboratory testing might be appropriate.

She doesn't endorse my choices. But she doesn't scold me either.

She respects my autonomy, and I respect her clinical judgment. That mutual trust means she has the information she needs to provide better care.

That's the partnership I'm asking you to consider.

My hope is that healthcare professionals will remain **informed enough to help patients navigate this emerging landscape safely, compassionately, and without judgment**.

You don't have to agree with our decisions.

You just have to be willing to understand them.

Thank you for taking the time to understand this emerging space. Your willingness to remain informed can make a profound difference in patient safety, openness, and trust.

With respect and appreciation,

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